Clinical Case Intake Form

For the reviewing oncologist — one form per new benchmark case

Fill in the fields below with the clinical ground truth for one synthetic case. Engineering will translate your answers into the evaluation system; you do not need to write code, JSON, or YAML, or look at the grader. Type directly into the boxes (this PDF is fillable) or print and write by hand.

A. Case Identification

Case ID (engineering can assign if unsure)
e.g. dlbcl_0006_cutaneous

Difficulty
beginner / intermediate / advanced

Case Title
Short description of what this case tests

Input format
narrative / FHIR / both

Cancer type / subdomain
e.g. DLBCL, follicular lymphoma, breast cancer

B. Synthetic Patient Vignette

Write the case exactly as the model should see it — this becomes the model's input.

History
Age, sex, presentation, symptoms, performance status (ECOG). No real patient data.

Labs
CBC, chemistry, LDH, and any other relevant labs with values and units.

Imaging
Modality (e.g. PET-CT) and findings: nodal sites, size, extranodal/organ involvement.

Pathology
Biopsy result, IHC markers with %, FISH rearrangement results, bone marrow status.

C. Gold Answer — one entry per required field

This is the answer the model's response will be graded against. Be specific and explicit; state values/results directly (e.g. "FISH negative for MYC/BCL2/BCL6") rather than implying them.

1. Diagnosis
Subtype, cell-of-origin call (e.g. GCB vs non-GCB by Hans) and whether that's definitive.

2. Stage
Ann Arbor / Lugano stage with the specific anatomic reasoning for that stage.

3. Biology / molecular classification
Double-expressor vs double-hit: state IHC overexpression values AND FISH result explicitly.

4. Prognosis
Which score(s) apply (e.g. IPI, CNS-IPI) and any data missing to complete them.

5. First-line treatment
The correct curative-intent regimen(s), and anything that would be over/under-treatment.

6. CNS risk
CNS-IPI-based risk level, high-risk anatomic sites if present, and whether prophylaxis is indicated.

7. Response assessment
Imaging modality/criteria to use, and at what treatment timepoint(s).

8. Missing information
Clinically relevant data not given in the case that would change the answer if known.

Synthetic training case for AI evaluation only. Not clinical decision support. Do not enter real patient-identifiable information.

D. Critical / Dangerous Errors

List specific wrong answers that should be treated as serious failures (not just lost points), e.g. “calls this double-hit despite a negative FISH” or “recommends non-curative treatment.”

1. Error description

Severity
minor/important/dangerous

2. Error description

Severity
minor/important/dangerous

3. Error description

Severity
minor/important/dangerous

4. Error description

Severity
minor/important/dangerous

E. Acceptable Alternative Answers

Other answers that should still receive full or partial credit (e.g. “R-CHOP accepted as an alternative to pola-R-CHP”).

Alternatives

F. Reviewer Sign-Off

Oncologist name

Credentials / title

Institution (optional)

Date
YYYY-MM-DD

Signature
Type name or sign if printing